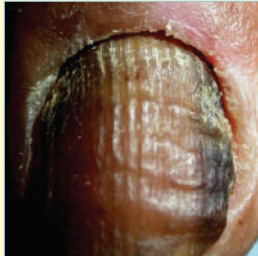


NAIL SYMPTOMS

- If nails long and dirty and patient unkempt, screen for mental health problem and abuse/neglect ➔ 86.
- Manage according to type of nail problem:

Disfigured nail with swollen nail bed and loss of cuticle



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Chronic paronychia likely

Usually associated with excessive exposure to water and irritants like nail cosmetics, soaps and chemicals.

- Advise to avoid water and irritants or to wear gloves if unavoidable. Keep hands clean and dry.
- After washing hands, massage **betamethasone 0.1%** cream into nailfold at night.
- If nailfold painful or pus, treat for infection:
 - Give **flucloxacillin** 500mg 6 hourly or **cefalexin** 500mg 6 hourly for 5 days.
 - If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days
- If no better, refer.

Pain, redness and swelling of nail folds, there may be pus.



© BMJ Best Practice

Acute paronychia likely

Often with history of trauma, such as nail biting, pushing the cuticle or cutting nails too short.

- Advise to avoid trauma to nail.
- If any pus, incise and drain.
- Give **flucloxacillin** 500mg 6 hourly or **cefalexin** 500mg 6 hourly for 5 days. If severe penicillin allergy¹, give instead **azithromycin** 500mg daily for 3 days.
- If no response, refer.

White/yellow disfigured or crumbling nails



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Fungal infection likely

- Test for HIV ➔ 110 and diabetes ➔ 17.
- Fungal nail infection is difficult to treat.
- If very distressing to patient, refer.

Blue/brown/black discolouration of nail



CDC Public Health Image Library

Has there been recent trauma to nail?

Yes

Haematoma likely

- Reassure patient.
- Treat if injury < 2 days old and painful:
 - Clean nail with **povidone iodine 10%** solution.
 - Hold finger secure and gently twist a large bore needle into nail over centre of haematoma. Stop when blood drains through hole.
 - Cover with sterile gauze dressing.

No

- Psoriasis may discolour nails. If psoriasis on skin ➔ 70.
- Review medication: fluconazole, ibuprofen, lamivudine, phenytoin and zidovudine can cause discolouration of nails. Discuss with doctor.
- Refer same week to exclude melanoma (picture above) if:
 - New dark spot on 1 nail which is getting bigger quickly and no recent trauma
 - Discolouration extends into nail folds
 - Band on nail that is:
 - > 4mm wide
 - Getting darker or bigger
 - Has blurred edges
 - Nail is damaged

Transverse dents in nails (Beau's lines)

- Check for paronychia in adjacent columns
- If above excluded, reassure likely due to previous illness/injury and will grow out with nail.

¹History of angioedema, anaphylaxis or urticaria.